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News Release

For immediate use July 12, 2004 -- No. 350

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Department of Defense funds consortium's research on racial, other disparities in prostate cancer death rates

CHAPEL HILL -- Researchers are preparing to begin a new study focused on why prostate cancer deaths are more than twice as common in black men as in white men and why such deaths also vary significantly from state to state.

A new consortium of top U.S. cancer researchers is leading the study, which is funded by a three-year, $9.9 million grant from the U.S. Department of Defense Prostate Cancer Research Program.

"Prostate cancer is the most common cancer in men and the second leading cause of cancer mortality in the United States," said Dr. James L. Mohler, consortium director.

Mohler is professor and chairman of the department of urologic oncology at Roswell Park Cancer Institute in Buffalo, N.Y. He remains a member of the University of North Carolina at Chapel Hill Lineberger Comprehensive Cancer Center and adjunct associate professor of surgery and pathology at UNC's School of Medicine, where he led the prostate cancer research program for 16 years.

"In men younger than age 65, the prostate cancer death rate for African Americans is 3.1 times that of Caucasian Americans. In men 65 and older, the prostate cancer mortality rate for African Americans is 2.3 times that of Caucasian Americans. Why that's true is an intriguing medical mystery that likely will hold clues to treating the deadly illness more successfully," said Mohler.

To try to solve it, investigators at Louisiana State University Health Sciences Center; Harvard, Johns Hopkins, Boston and Wake Forest universities; the universities of South Carolina and California at Irvine; and Roswell Park Cancer Institute will join Mohler and other scientists at the National Cancer Institute, the National Institute of Environmental Health Sciences and the U.S. Food and Drug Administration as one of the two newly funded Prostate Cancer Consortiums.

"One of our goals is to study 2,000 patients with newly diagnosed prostate cancer," said Mohler. "We will interview 500 African-American men and 500 Caucasian-American men in both North Carolina and Louisiana and collect and analyze both blood and fat samples from the subjects.

"Men will be recruited for this study," he added, "so we are not asking for volunteers, but we hope that if a man is called to take part, he will agree to help us with this important study."

The team is focusing on the two states because North Carolina often has the highest prostate cancer incidence and death rates nationwide for black men, while Louisiana has one of the lowest, said Dr. Elizabeth T. Fontham, leader of Louisiana's consortium efforts. Fontham is dean of the school of public health, professor of pathology and associate director of the Stanley Scott Cancer Center at Louisiana State University Health Sciences Center.

The two states have similar incidence and mortality rates for white men, however.

Three reasons have been suggested for the disproportionate mortality between the two races, Mohler said. "First, African Americans may present more often with advanced, incurable prostate cancer because of more limited access to health care. African Americans have been reported more likely to just let the disease follow its course, which doctors often advise in men over age 75."

Second, biological differences between the two races may cause prostate cancer to develop at a younger age or grow and spread more rapidly in blacks, Mohler said. "Finally," he added, "the prostate cancers that occur in African Americans may be inherently more aggressive. These studies will help pinpoint which of these three categories are important."

Researchers at the participating institutions have particular expertise they can bring to bear on the questions, he said, and that's why they are working together. One major result will be an invaluable central resource of clinical and research data on prostate cancer patients, Mohler said, eventually including what happens to patients following various treatments.

Another result should be a better understanding of what can be done to reduce prostate cancer deaths in general and in the African-American population specifically. "These studies should demonstrate whether public health resources should be focused on altering interactions between patients and the health-care system, changing diets or altering patient or tumor biology," Mohler said.

The other consortium is based at Emory University in Atlanta.

This year, more than 30,000 men nationwide will die from prostate cancer, American Cancer Society statistics show, and close to 190,000 new cases will be diagnosed.

The prostate gland is a chestnut-shaped male organ surrounding the urethra just below the bladder. Its purpose is to produce secretions that keep the lining of the urethra moist and others that form part of the seminal fluid. The prostate grows during puberty and begins to enlarge further in most men after age 50, sometimes interfering with urination.

Cancers increase in frequency as men age, research shows. Early detection using prostate examination and blood tests for prostate-specific antigen, or PSA, can detect the disease before symptoms develop when the cancer is most curable. Treatment options include "watchful waiting," radiation, operation and hormonal therapies.

Note: Mohler will be in Chapel Hill today (July 12) and available for interviews. To arrange an interview, contact Dianne Shaw at (919) 966-5905 or dgs@med.unc.edu. He may be reached by phone, (716) 713-6700, after that date. Dr. Jeannette Bensen, study coordinator, also is available for interviews and may be reached at (919) 843-1017. Study participants and advocates are available for interviews.

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